

DESIGN ADDENDUM

Reviewer Identity, Credentials & Accountability

Surgery Authorization AI App — September 3, 2026

Purpose

This addendum adds a reviewer-identification and accountability layer to the peer-to-peer and utilization-review workflow. A recurring operational problem is that a reviewing physician may not voluntarily provide a complete name, license number, specialty, or other identifying information during a peer-to-peer. If the review later appears materially deficient, incomplete identification can make it difficult to verify reviewer qualifications or submit a focused complaint to the appropriate oversight body.

1. Mandatory Peer-to-Peer Identity Intake

At the beginning of every peer-to-peer, the app should prompt the surgeon or staff to request and record:

- Full name of reviewing physician
- Degree (MD/DO or other credential)
- Specialty and subspecialty
- State(s) of medical licensure
- Medical license number
- Utilization-review company / independent review organization, if applicable
- Carrier or health plan represented
- Call date and time
- Case / authorization / claim number

2. Suggested Opening Script

“Before we discuss the case, for my documentation, could you please provide your full name, degree, specialty/subspecialty, state of licensure, and medical-license number?”

The request should be routine and non-confrontational. The app should not imply that every jurisdiction requires the physician to verbally disclose all of this information during the call.

3. If the Reviewer Refuses

The app should capture the refusal rather than encouraging an argument during the peer-to-peer.

Reviewer declined to provide [full name / license number / specialty / other requested credential] when requested.

Then prompt the surgeon to ask:

“Please tell me how I may obtain the written utilization-review or peer-review determination identifying the reviewing physician and the reviewer’s credentials.”

4. Legal / Jurisdiction Rule Engine

There is no single nationwide rule that should be assumed to require every peer-to-peer physician to verbally provide both full name and license number during the call. The application must determine the governing plan type and jurisdiction before asserting a disclosure requirement.

- Texas workers' compensation: build a specific rule set for peer-review report requirements, including reviewer identity/license and documentation of records reviewed when applicable.
- State-regulated commercial plans: retrieve the applicable state utilization-review, adverse-determination, peer-review, and reviewer-qualification requirements.
- Self-funded/ERISA plans: identify the applicable federal plan/claims-review requirements and avoid applying state insurance rules that do not govern the plan.
- Other workers' compensation jurisdictions: retrieve that state's WC utilization-review and peer-review rules independently.
- Never convert a jurisdiction-specific requirement into a universal app rule.

5. Reviewer Identity States

The GUI should distinguish identity quality rather than simply storing a name:

- SELF-IDENTIFIED — information supplied by the reviewer during the call.
- DOCUMENT-IDENTIFIED — identity extracted from written peer-review/adverse-determination materials.
- INDEPENDENTLY VERIFIED — name/license matched against the applicable state licensing database or other authoritative source.
- PARTIALLY IDENTIFIED — e.g., first name/last initial or name without license number.
- IDENTITY REQUESTED — REVIEWER DECLINED — refusal documented.
- IDENTITY PENDING — written determination/report requested.

6. Independent Credential Verification

When sufficient identifying information is available, the app should support verification against authoritative public licensing sources. The objective is not merely to know who conducted the review, but to determine whether the reviewer appears appropriately qualified for the clinical issue under review.

- Verify active license and jurisdiction.
- Capture specialty/subspecialty when available from an authoritative source.
- Compare reviewer specialty with requested procedure and treating surgeon specialty.
- Preserve source and verification date.
- Do not infer board certification or spine-specific expertise merely from a general 'Orthopaedic Surgery' label.
- Flag qualifications that require human review rather than automatically alleging inadequacy.

7. Link Reviewer Identity to the Evidence Chain

This module should connect directly to the Submission & Receipt Verification addendum. The system should be able to answer three questions for every adverse review:

WHAT did our office transmit?

WHO reviewed the request?

WHAT did that reviewer say was actually available/reviewed?

If a reviewer says, for example, that physical-therapy records or an MRI were not available, the app should immediately compare that statement with the submission manifest, transmission evidence, carrier receipt response/nonresponse, and the eventual written peer-review report.

8. Peer-to-Peer Call Record

Create a structured contemporaneous call log containing:

- Reviewer identity/credential fields and identity status
- Exact policy/pathway discussed
- Documents reviewer states were reviewed
- Documents reviewer states were missing
- Material statements affecting approval/denial
- Whether reviewer applied a different policy section than the one submitted
- Whether reviewer requested additional information
- Whether additional records were resent during or immediately after the call
- Outcome of the call
- Any refusal to identify or provide credential information

9. Written Peer-Review Report Retrieval

When the reviewer will not provide complete identifying information verbally, the workflow should automatically create a task to obtain the written peer-review/adverse-determination report and extract the reviewer identity, credentials, rationale, and list of records reviewed where required or available.

For jurisdictions in which the written report is required to identify the reviewer or enumerate records reviewed, the app should cite the applicable rule in the staff task and preserve the report as part of the case's accountability record.

10. Complaint Trigger — Review, Not Automatic Accusation

The app should never automatically accuse a reviewer of misconduct. Instead, it should create a potential accountability event when facts warrant human review.

- Reviewer refuses identifying information and the written determination also fails to provide required identification.
- Reviewer specialty appears materially mismatched to the requested procedure.
- Reviewer claims critical records were absent despite documented transmission/receipt evidence.
- Written report omits records that the carrier previously confirmed were in the review file.
- Reviewer applies a materially different policy pathway from the submitted indication without addressing the selected pathway.
- Required physician-to-physician discussion or specialty review was promised but not provided.
- Other documented utilization-review process deficiencies materially affect the determination.

11. Regulatory Complaint Package Generator

If the surgeon elects to escalate, the app should assemble a factual complaint package rather than forcing staff to reconstruct the history.

- Patient/case identifiers appropriate to the complaint process

- Carrier / UR organization
- Reviewer name, license number, specialty, and verification source when known
- Chronology of authorization, submissions, denials, and peer-to-peer contacts
- Submission manifests and transmission confirmations
- Receipt-verification correspondence
- Peer-to-peer call log
- Written peer-review/adverse determination
- Policy section submitted versus policy section applied
- Specific factual discrepancy or process concern
- Applicable regulator and jurisdictional basis

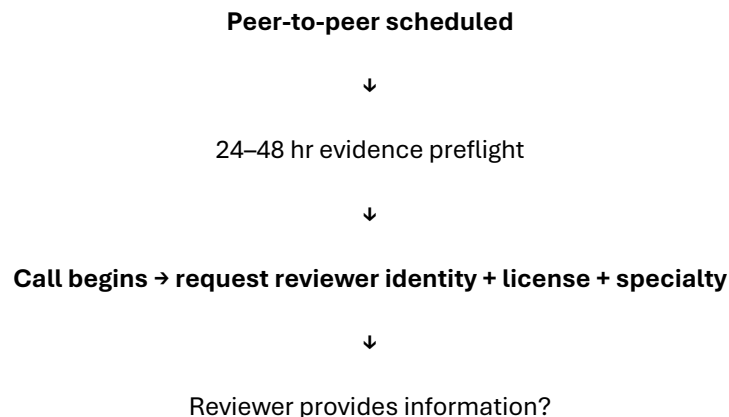
12. GUI Additions

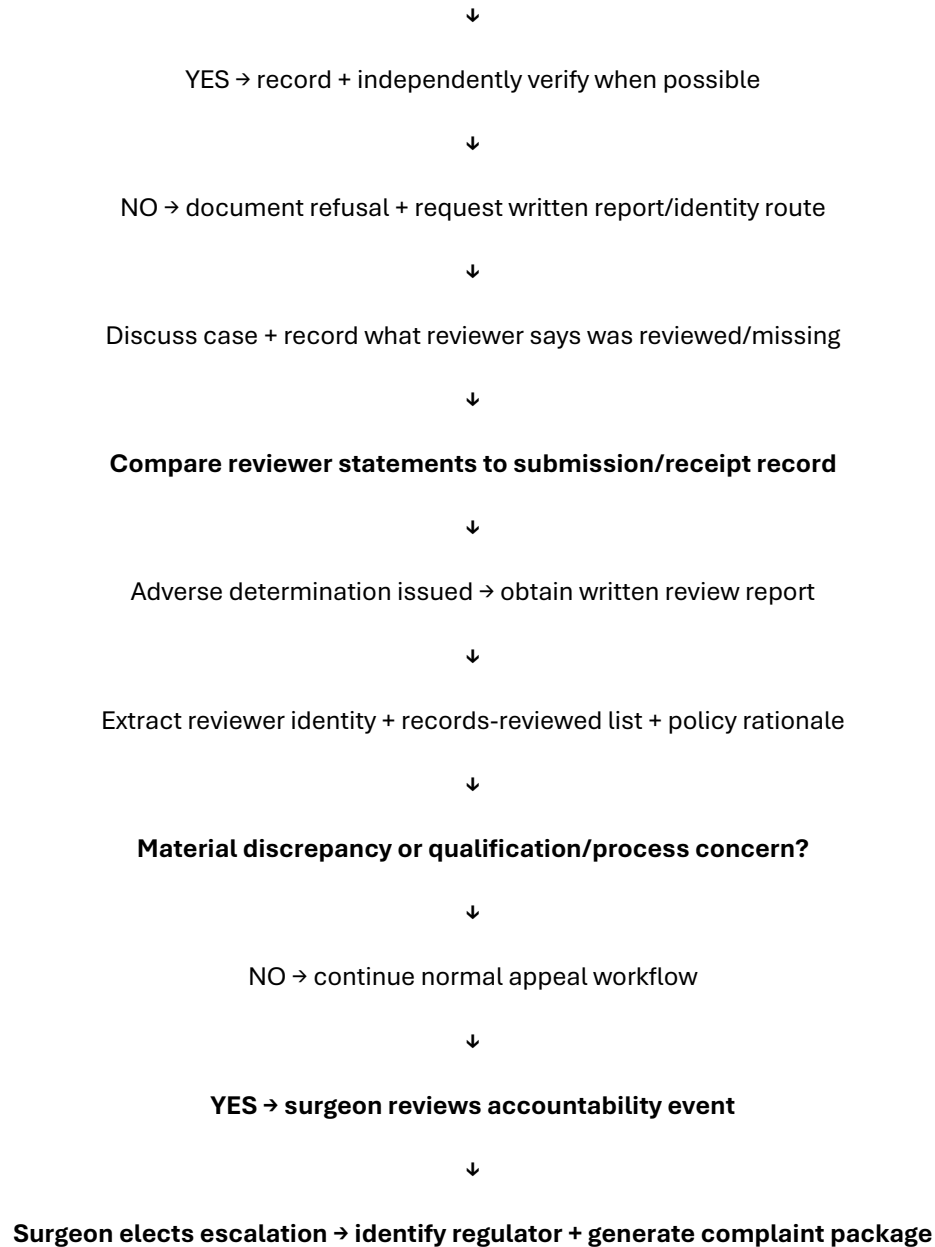
- Reviewer Identity card at the top of every Peer-to-Peer screen.
- One-tap 'Reviewer declined' controls for name, license, specialty, or organization.
- Credential Verification panel with source/date/status.
- Records Reviewer Says Available checklist.
- Records Reviewer Says Missing checklist linked to the submission manifest.
- Written Report Requested / Received status.
- Potential Accountability Event banner requiring surgeon review.
- Complaint Package button enabled only after jurisdiction and supporting evidence are established.

13. Developer Data Objects

- ReviewerIdentity: full name, degree, specialty, subspecialty, states, license numbers, organization, carrier, identity source/status.
- CredentialVerification: authoritative source, lookup date, license status, specialty data, verification confidence.
- PeerToPeerCall: timestamp, participants, case ID, reviewer statements, documents reviewed/missing, outcome.
- IdentityRefusalEvent: field refused, timestamp, exact/near-exact statement, follow-up request.
- WrittenReviewReport: report date, reviewer identity, rationale, policy/pathway, records-reviewed list, source file.
- AccountabilityEvent: discrepancy type, evidence links, materiality, surgeon disposition.
- ComplaintPackage: jurisdiction, regulator, factual chronology, exhibits, reviewer credentials, submission evidence.

14. Updated Peer-to-Peer Workflow Diagram





15. Peer-to-Peer Conversation Recording & AI Transcription

The app should support recording the actual peer-to-peer conversation when legally permitted. A recording is substantially more reliable than reconstructing the discussion afterward and can allow the AI to create a contemporaneous transcript, structured call summary, and accountability record.

Because call-recording consent laws vary by state and may depend on the locations of all participants, the recording function must be jurisdiction-aware. The app should not silently record calls. Before recording begins, it should determine the applicable consent rule when possible and require the surgeon to use an appropriate disclosure/consent workflow.

Recommended recording workflow:

- Before call: identify the likely jurisdictions of the surgeon and reviewer and determine whether recording requires one-party or all-party consent.
- Display a clear RECORDING PERMITTED / CONSENT REQUIRED / LEGAL STATUS UNCERTAIN indicator.

- When consent or disclosure is required, provide an on-screen script and require affirmative confirmation before recording.
- Capture the complete audio call when permitted, including reviewer identity discussion, clinical discussion, records-availability statements, policy discussion, and final disposition.
- Store the original audio as an immutable source artifact with date/time, case number, participants, and recording-consent metadata.
- Generate an AI transcript linked to timestamps in the audio.
- Generate a structured peer-to-peer summary from the transcript, but preserve the audio/transcript as the source of truth.
- Allow the surgeon to correct transcription errors without altering or overwriting the original recording/transcript.

AI extraction from the recorded call:

- Reviewer full name, degree, specialty/subspecialty, state and license number if stated.
- Whether the reviewer refused to provide identifying information.
- Policy name/section or criteria discussed.
- Documents the reviewer states were available or reviewed.
- Documents the reviewer states were missing or unavailable.
- Clinical findings accepted, disputed, or misunderstood by the reviewer.
- Additional information requested.
- Reason given for approval or denial.
- Statements materially inconsistent with the written adverse determination.
- Follow-up commitments made by the reviewer or carrier.

Recording / transcript status fields:

- NOT RECORDED — recording not initiated.
- RECORDING LEGALLY PERMITTED — captured.
- CONSENT OBTAINED — captured after required consent.
- REVIEWER DECLINED RECORDING — no audio; structured contemporaneous notes required.
- LEGAL STATUS UNCERTAIN — recording disabled pending clarification.
- TRANSCRIPT GENERATED — AI transcript available.
- SURGEON REVIEWED — transcript/summary reviewed for material accuracy.

Suggested disclosure when consent is required:

“For accuracy and documentation of this utilization-review discussion, I would like to record this peer-to-peer conversation. Do I have your consent to record the call?”

If the reviewer declines, the app should immediately switch to enhanced contemporaneous documentation mode and capture the reviewer identity request, records discussed, policy discussion, material statements, and outcome. A refusal to be recorded should not itself be treated as evidence of wrongdoing.

Recording-related developer objects:

- CallRecording: case ID, audio artifact, start/end timestamps, participants, jurisdictions, consent status, disclosure text, immutable-source hash/reference.
- CallTranscript: recording ID, timestamped transcript, AI confidence, corrections/annotations, surgeon-review status.
- CallExtraction: reviewer identity, policy references, records reviewed/missing, rationale, commitments, outcome, transcript timestamps.
- RecordingConsentEvent: participant, jurisdiction, consent requirement, disclosure made, response, timestamp.

- TranscriptDiscrepancy: recorded statement versus later written determination, materiality, supporting timestamps, surgeon disposition.

The complaint-package generator should be able to include the recording or relevant transcript excerpts when legally appropriate and useful, while preserving the complete original recording. The app should never selectively alter audio or present an AI summary as though it were a verbatim transcript.

16. Product Outcome

Combined with the prior design and Submission & Receipt Verification addendum, the application should preserve a complete Utilization Review Accountability Record: the exact policy pathway submitted, the evidence transmitted, proof of transmission/receipt status, the identity and qualifications of the reviewing physician, what that physician states was reviewed, the written rationale, and any material discrepancies.

WHAT WE SENT → WHO REVIEWED IT → WHAT THEY REVIEWED → WHY THEY DENIED IT